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Submitted via ASAM public-comment survey

American Society of Addiction Medicine (ASAM)
Editorial Team — The ASAM Criteria, Volume 3
Correctional Settings & Community Reentry Standards
Public comment survey

RE: Public comment on Correctional Settings & Community Reentry Standards (The ASAM Criteria, Volume 3)

Dear ASAM Editorial Team:

On behalf of the Society of Addiction Medicine Physician Associates (SAMPA), thank you for the opportunity to comment on the draft Correctional Settings & Community Reentry Standards. SAMPA is the national professional organization representing physician associates/physician assistants (PAs) who practice in addiction medicine. Our members deliver evidence-based care for substance use disorders across hospitals, primary care, specialty clinics, and carceral and reentry settings.

The central message of these comments is an access and continuity message: standards should keep verified medications for opioid use disorder (MOUD) moving at intake, transfer, and release; use practitioner-neutral language for prescribers and telehealth; close methadone carve-outs that become permanent exemptions; distinguish jail/prison health-system medical directors (state law) from OTP medical directors (physician under 42 CFR §8.2); and treat peer/recovery support as complementary to MOUD—not a substitute or contingency. Naming PAs and other APPs as prescribers is how more people stay on treatment in APP-staffed facilities — a standard they cannot run is a standard that stops MOUD.

Citations use a single page and line from the official ASAM PDF, shown as small rounded capsules in the form [P63 L24]. [Download the PDF.](#)

Submitted comments

[P11 L5] 1. Intake: continue verified MOUD; keep “prescriber”

Guideline: *Verified medications (including medications for OUD) should be continued unless otherwise ordered by a prescriber based on documented clinical need.*

Comment: Strongly support uninterrupted continuation of verified MOUD at intake. Please keep “prescriber” (not physician-only) as the authority to modify therapy, consistent with BJA/NIC jail withdrawal guidance that uses “physician” only when a physician is required and recognizes that NPs/PAs may diagnose and initiate treatment. Forced taper at booking drives post-release overdose risk and is operationally unworkable in APP-staffed jails.

[P11 L8] 2. Intake: keep physician + APP pairing throughout Volume 3

Guideline: *If staff are unable to verify the prescription prior to the next scheduled dose of the medication, the physician or advanced practice provider should be notified and they should make the determination of whether the medication should be continued pending verification.*

Comment: Appreciate physician + APP pairing here. Please keep this inclusive throughout Volume 3 wherever medication continuation, induction, or telehealth medication management is described—do not narrow later sections to physician-only.

[P11 L12] 3. Custody-assisted screening: no authority to deny MOUD

Guideline: *Editorial Team asks about “well-trained and supervised custody staff conducting health screenings when health care professionals are not present.”*

Comment: Custody-assisted screening may be a pragmatic bridge in understaffed jails ONLY with standardized protocols, immediate escalation pathways, and no authority to discontinue or deny MOUD. Prefer qualified health professionals whenever available. If retained, state explicitly that custody screening cannot delay verified MOUD continuation or APP/physician assessment for withdrawal.

Suggested revision: If well-trained and supervised custody staff conduct intake health screenings when health care professionals are not present, such screening must follow standardized protocols with immediate escalation to a physician or advanced practice provider. Custody staff shall have no authority to discontinue, deny, or delay verified MOUD, and custody screening shall not delay continuation of verified addiction medications or APP/physician assessment for withdrawal. Qualified health professionals should conduct intake health screenings whenever available.

[P15 L2] 4. Telemedicine: practitioner-neutral, not specialty-physician-only

Guideline: *...at risk for withdrawal when entering the facility. Consideration of addiction medication needs may occur through telemedicine.*

Comment: Please affirm practitioner-neutral telemedicine for addiction medication assessment/management (physicians and APPs with appropriate DEA/state authority), not specialty-physician-only telehealth. Permanent SUD telehealth flexibilities with practitioner-neutral language are a core SAMPA policy priority—especially for rural and understaffed jails.

Suggested revision: Consideration of addiction medication needs may occur through telemedicine with qualified practitioners (physicians and advanced practice providers with appropriate DEA registration and state authority).

[P32 L32] 5. Reentry barrier: limited technology should not justify withholding telehealth

Guideline: *Limited access to technology (eg, for telehealth or online support groups) — listed as a reentry barrier.*

Comment: Good catch as a reentry barrier. Standards should push facilities/partners to solve telehealth access (device, privacy, broadband) rather than treat limited technology as an accepted reason to withhold remote MOUD/psychosocial care. Practitioner-neutral telehealth is a SAMPA priority.

[P35 L14] 6. Methadone “unless unavailable locally” carve-out

Guideline: *** Unless unavailable locally. (note that the standard of care for opioid withdrawal management includes treatment with buprenorphine or methadone.)*

Comment: “Unless unavailable locally” risks becoming a permanent methadone carve-out. Prefer requiring facilities to ensure access via on-site OTP/medication unit, hospital/clinic pathway, community OTP partnership, or transport—consistent with ASAM correctional OUD policy and Part 8 practitioner models (PAs/NPs as OTP practitioners where state law allows; Medical Director remains a physician under §8.2).

Suggested revision: *** Facilities must ensure access to all FDA-approved medications for OUD via on-site OTP/medication unit, hospital/clinic pathway, community OTP partnership, or transport; local unavailability requires a documented access plan, not an exemption from the standard of care.*

[P39 L16] 7. Jail medical director: state-law APP eligibility; OTP MD remains physician

Guideline: *The medical director, who oversees medical care for the jail system, should be a physician with controlled substance prescribing authority.*

Comment: Distinguish jail health-system medical director from OTP medical director. A SAMHSA-certified OTP medical director must be a physician (42 CFR §8.2). For jail health services, medical director eligibility varies by state law and program type—in some jurisdictions credentialed NPs and PAs may serve as medical director where state licensing rules allow (eg, Illinois permits PA/NP medical directors for certain SUD levels, but not OTP).

Suggested revision: The medical director, who oversees medical care for the jail system, should be a licensed clinician with controlled substance prescribing authority who meets applicable state licensing and accreditation requirements; where state law permits, this may be a physician, nurse practitioner (NP), or Physician Associate/Assistant (PA). Where the jail operates or partners with a SAMHSA-certified opioid treatment program (OTP), the OTP medical director must be a physician (42 CFR §8.2). The medical director may delegate clinical MOUD responsibilities to credentialed NPs and PAs with prescriptive authority consistent with applicable federal and state law.

[P40 L25] 8. Jail allied health / peers: expected access alongside MOUD

Guideline: *Jails may have allied health staff (eg, peer support specialists, patient navigators, non-clinical case managers, etc.) who support individuals who have SUD...*

Comment: Support explicit peer/recovery support staffing. Please strengthen “may have” toward expected access to peer recovery services alongside MOUD (not as a substitute for medication).

Suggested revision: Jails should have allied health staff (eg, peer support specialists, patient navigators, non-clinical case managers, etc.) who support individuals who have SUD and co-occurring mental health conditions alongside MOUD (not as a substitute for medication).

[P47 L21] 9. Jail RSS required: MOUD not contingent on counseling/RSS

Guideline: *All jails should provide RSS (eg, mutual support programs, peer support, vocational support) either directly or through formally affiliated organizations...*

Comment: Strongly support required RSS/peer supports. Please clarify MOUD is not contingent on counseling/RSS participation (offer concurrently). Low-barrier medication + peers is central to PA patient advocacy in carceral and reentry settings.

Suggested revision: All jails should provide RSS (eg, mutual support programs, peer support, vocational support) either directly or through formally affiliated organizations or providers (eg, certified peer support specialists, recovery community organizations [RCOs], recovery community centers [RCCs]) for individuals receiving SUD treatment services. RSS and counseling should be offered concurrently with MOUD and must not be a condition of receiving addiction medications.

[P52 L8] 10. Jail staffing: “physician supervisor” bottleneck

Guideline: *The number of physicians and advanced practice providers the medical director or other physician supervisor can safely manage*

Comment: “Physician supervisor” staffing-ratio language may be read to exclude PA/NP supervisory and collaborative practice models permitted under state law.

Suggested revision: The number of physicians and advanced practice providers the medical director or other authorized clinical supervisor (physician or advanced practice provider, as applicable under state law) can safely manage

[P63 L21] 11. Jail reentry resources: name PA-accessible nodes

Guideline: *...home community (eg, OTPs, withdrawal management services, bridge clinics, crisis services)*

Comment: Please name PA-accessible reentry nodes explicitly (bridge clinics, office-based MOUD, OTPs). Warm handoff should be to any eligible community MOUD practitioner—including PAs—not physician-appointment-only. Day-of-release gaps drive overdose mortality.

Suggested revision: Written information on relevant treatment and recovery supportive resources in their home community (eg, OTPs, medication units, bridge clinics, office-based MOUD programs, withdrawal management services, crisis services), with warm handoff to any eligible MOUD practitioner—including physicians and advanced practice providers

[P64 L30] 12. Jail release: clinically adequate bridge + timely follow-up

Guideline: *Access to a clinically appropriate amount of the patient's prescription medications, including addiction medications (if applicable), as a bridge until the patient can follow up with community MOUD practitioners (including physicians and advanced practice providers)*

Comment: Strongly support bridge supply of addiction medications at release. Please specify a clinically adequate bridge (not token doses)—eg, a 1-month supply, or a monthly long-acting injectable buprenorphine (LAIB) administered prior to release—and same-week follow-up with a community MOUD practitioner (physician or APP). This is high-impact for PA-led reentry clinics.

Suggested revision: Access to a clinically appropriate amount of the patient's prescription medications, including addiction medications (if applicable)—eg, a 1-month supply, or a monthly long-acting injectable buprenorphine (LAIB) administered prior to release—as a bridge until the patient can follow up with community MOUD practitioners (including physicians and advanced practice providers), with that follow-up scheduled no later than 7 days post release (or sooner if clinically indicated)

[P65 L1] 13. Jail methadone linkage: OTP coordination + practitioner-neutral clinicians

Guideline: *Linkage to a community pharmacy or methadone treatment provider as applicable to facilitate sharing of medication records and provision of medication*

Comment: Support linkage language. For methadone, specify OTP/medication-unit coordination and record-sharing so receiving practitioners (including PAs/NPs practicing in OTPs where allowed) can continue without forced interruption. Align with Part 8 practitioner definition and SAMHSA prison MOUD implementation guidance.

Suggested revision: Linkage to a community OTP or medication unit (for methadone), pharmacy, or other MOUD provider as applicable to facilitate sharing of medication records and provision of medication, so receiving practitioners—including physicians and advanced practice providers authorized under applicable federal and state law—can continue treatment without forced interruption

[P70 L3] 14. Prison medical director: same as jail (state law; OTP MD = physician)

Guideline: *The medical director, who oversees medical care for the prison system should be a physician with controlled substance prescribing authority.*

Comment: Same rationale as jail medical director (p.039). Distinguish prison health-system medical director from OTP medical director. A SAMHSA-certified OTP medical director must be a physician (42 CFR §8.2). For prison health services, medical director eligibility varies by state law and program type—in some jurisdictions credentialed NPs and PAs may serve as medical director where state licensing rules allow (eg, Illinois permits PA/NP medical directors for certain SUD levels, but not OTP).

Suggested revision: The medical director, who oversees medical care for the prison system, should be a licensed clinician with controlled substance prescribing authority who meets applicable state licensing and accreditation requirements; where state law permits, this may be a physician, nurse practitioner (NP), or Physician Associate/Assistant (PA). Where the prison operates or partners with a SAMHSA-certified opioid treatment program (OTP), the OTP medical director must be a physician (42 CFR §8.2). The medical director may delegate clinical MOUD responsibilities to credentialed NPs and PAs with prescriptive authority consistent with applicable federal and state law.

[P82 L32] 15. Prison staffing: same “physician supervisor” concern

Guideline: *The number of physicians and advanced practice providers the medical director or other physician supervisor can safely manage*

Comment: Same rationale as jail staffing (p.052). “Physician supervisor” language may exclude APP supervisory and collaborative practice models permitted under state law.

Suggested revision: The number of physicians and advanced practice providers the medical director or other authorized clinical supervisor (physician or advanced practice provider, as applicable under state law) can safely manage

[P93 L1] 16. Prison methadone linkage: same as jail reentry

Guideline: *Linkage to a community pharmacy or methadone treatment provider as applicable to facilitate sharing of medication records and provision of medication*

Comment: Same rationale as jail methadone linkage (p.065). Support linkage language. For methadone, specify OTP/medication-unit coordination and record-sharing so receiving practitioners (including PAs/NPs practicing in OTPs where allowed) can continue without forced interruption. Align with Part 8 practitioner definition and SAMHSA prison MOUD implementation guidance.

Suggested revision: Linkage to a community OTP or medication unit (for methadone), pharmacy, or other MOUD provider as applicable to facilitate sharing of medication records and provision of medication, so receiving practitioners—including physicians and advanced practice providers authorized under applicable federal and state law—can continue treatment without forced interruption

[P95 L23] 17. Adolescents: APP consultation when specialist physician unavailable

Guideline: *Addiction specialist physicians with at least 2 years of experience treating adolescents with SUD available as needed for consultation*

Comment: Specialty physician consultation is valuable, but please also allow consultation from APPs with adolescent SUD competency and appropriate scope when a specialist physician is unavailable. Patients should not lose MOUD access due to specialist scarcity.

Suggested revision: Addiction specialist physicians with at least 2 years of experience treating adolescents with SUD available as needed for consultation; where a specialist physician is unavailable, consultation may also be provided by advanced practice providers with adolescent SUD competency and appropriate scope under state law

Questions about this submission may be directed to the SAMPA Public Health Policy Committee at policy@addictionpas.org. SAMPA’s broader access priorities are published at <https://www.addictionpas.org/policy>.

Respectfully submitted,

Society of Addiction Medicine Physician Associates (SAMPA)